

**REFER NOW
SUSPECTED LASSA FEVER****PATIENT**

Age: 35 years
State / LGA: Ondo

Sex: —

PRESENTING FINDINGS

- Fever for 5 day(s)
- Malaria RDT: negative
- Headache
- Sore throat

TRIAGE DECISION**REFER NOW**

Rule floor is refer_now; patient has fever >48hrs, sore throat, headache, no response to antimalarial, and Ondo has active Lassa fever outbreak, raising suspicion for VHF requiring urgent referral.

SUSPECTED CONDITION

1. Lassa fever (*high*)
2. Streptococcal pharyngitis (*moderate*)
3. Typhoid fever (*low*)

ADVISED ACTIONS

1. Suspected Lassa fever: keep the patient in a separate holding area, use infection prevention measures (gloves; avoid contact with blood and body fluids), notify the LGA disease surveillance officer, and refer for Lassa testing.
2. Alert the relevant authorities: infectious disease team or responsible physician
3. Start rehydration. Commence Fluid therapy (see guidance on fluid replacement in Chapter 4)
4. Monitor vital signs every 4 hours (Pulse Rate, Blood Pressure, Respiratory Rate, Temperature)

OUTBREAK CONTEXT

- Lassa fever — Ondo (live report, 2026-09-06)
- Lassa fever — Ondo (NCDC Lassa Fever Situation Report, Week 36, 2026, p.1)
- Cerebrospinal meningitis — Ondo (NCDC Cerebrospinal Meningitis Situation Report, Week 14, 2025 (cumulative Epi week 40 2024 - 14 2025), p.1 (older surveillance))
- Cholera — Ondo (NCDC Cholera Situation Report, Week 9, 2025, p.6 (older surveillance))

TO BE COMPLETED BY HEALTH WORKER

Referred to (facility): _____

Referred by (name and cadre): _____

Date: _____ Time: _____ Phone: _____

Notes: _____

Decision support only, not a diagnosis. A qualified health worker must confirm every decision. Doses come from the guideline table only.

Generated by Ibà — decision support only. Not a diagnosis.